

CHAROTAR UNIVERSITY OF SCIENCE AND TECHNOLOGY

Third Semester M.Sc Nursing Examination

March-2019

Course Code & Name: NR 831 OBSTETRIC AND GYNAECOLOGICAL NURSING-III

Date & Day: 20.03.2019, Wednesday

Time: 10:00 a.m. to 10:30 a.m.

Maximum Marks: 15

SECTION-I

Q.1. Multiple choice question: [Tick ✓ correct answer in given bracket]

15x1=15

1. A G₂P₁A₀ woman had her last menstrual period 9 weeks ago & now she presents with bleeding, passage of small bits of tissues per vagina & lower abdominal pain. The most likely diagnosis is
 - a. Inevitable abortion []
 - b. Incomplete abortion []
 - c. Ectopic pregnancy []
 - d. Multiple pregnancy []
2. The common possible etiology of oligohydramnious is
 - a. Diabetes mellitus []
 - b. Rh incompatibility []
 - c. Chorioangioma of placenta []
 - d. Fetal renal agenesis []
3. Per Vaginal examination is contraindicated in cases diagnosed with.....
 - a. Prolapsed cord []
 - b. Placenta praevia []
 - c. Gonorrhoea []
 - d. HELLP syndrome []
4. A 33 year old G₅P₄A₀L₄ woman presented to antenatal clinic with complains of weakness & is diagnosed to have Anemia. The most likely type of anemia in her case is
 - a. Thalassemia []
 - b. Pernicious anemia []
 - c. Iron deficiency anemia []
 - d. Sick cell anemia []
5. In neonates, lack of surfactant related to Respiratory Distress Syndrome (RDS) is measured with diagnostic finding.
 - a. L/S ratio of 1:1 []
 - b. Respiration rate of 60/minute []
 - c. Collapsed bronchioles []
 - d. Chest X-ray []
6. As per WHO in "risk approach" used for managerial tool for MCH care, a gravid above age of 35 years & the gravid with convulsions during current pregnancy is categorized under
 - a. Category 1 []
 - b. Category 2 []
 - c. Category 3 []
 - d. Category 4 []
7. In triple test, abnormal level of hCG and estriol indicate that the developing fetus may have.....
 - a. Trisomy 21 []
 - b. Open neural tube defect []
 - c. IUGR fetus []
 - d. Genetic disorder []

8. In the diagnosis of HIV in pregnancy, is used as a screening test.
 - a. Enzyme immunoassay []
 - b. Polymerase chain reaction []
 - c. Western blot []
 - d. Immunofluorescence assay []
9. Fetal death before 20 weeks gestation without expulsion of any fetal/maternal tissue for at least 8 weeks thereafter is seen in
 - a. Missed abortion []
 - b. Gestational trophoblastic disease []
 - c. Septic abortion []
 - d. ABO incompatibility []
10. In Rh factor, type of antibodies crosses the placenta and causes damage to the fetus.
 - a. IgM []
 - b. IgA []
 - c. IgG []
 - d. IgF []
11. A new born presenting with extreme prematurity, critical illness and needing surgical intervention should be treated at NICU services.
 - a. Level-I []
 - b. Level-II []
 - c. Level-III []
 - d. Level-IV []
12. In case of full term vaginal delivery of twin pregnancy, the injection oxytocin is given after the delivery of
 - a. First twin []
 - b. Second twin []
 - c. First placenta []
 - d. Second placenta []
13. In type of abnormal uterine action, the law of normal polarity is maintained.
 - a. Uterine inertia []
 - b. Colicky uterus []
 - c. Spastic lower uterine segment []
 - d. Uterine tetany []
14. In case of atonic PPH, the cardinal drug in the management is
 - a. Oxytocin []
 - b. Methargin []
 - c. Misoprostol []
 - d. 15 methyl PGF2 alpha []
15. In type of postnatal psychiatric disorders, the nurse should refrain the mother from breastfeeding, if she is on treatment.
 - a. Puerperal blues []
 - b. Postpartum depression []
 - c. Postpartum psychosis []
 - d. None of the above []

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Date & Day: 20.03.2019, Wednesday

Time: 10:30 a.m. to 1:00 p.m.

Maximum Marks: 60

Instructions:

- Write each section in separate booklet.
- Draw neat labelled diagrams wherever necessary.

SECTION-II

Q.2. Mrs. Geet, a primigravida with 22 weeks of gestation, presents with complaints of continuous severe headache, occasional blurring of vision & severe swelling in her legs, face & hands. On initial diagnostic evaluation, you find her BP to be 160/100 mm of Hg & increased protein in her urine but no episode of convulsion. Answer the following in relation to this case:

- What would be the most likely diagnosis of Mrs. Geet. **[1+3+6]**
- Enumerate possible risk factors & etio-pathogenesis associated with Mrs. Geet's disease condition.
- Construct a detailed nursing care plan for Mrs. Geet.

- Q.3.**
- Write about Ophthalmia Neonatorum. **[5+5]**
 - Define Preterm infant and write a note about the characteristic features of a preterm infant.

OR

Q.3. Write in detail about Multiple pregnancy. **[10]**

Q.4. Answer **Any Two** of the following: **[2x5]**

- Role of Nurse in prevention and management of birth injuries.
- Intra Uterine Growth Retardation (IUGR) - Definition, etiology and differentiate between Symmetrical & Asymmetrical IUGR.
- Rh incompatibility.

SECTION-III

- Q.5.**
- Define Postpartum Haemorrhage (PPH). **[1+3+6]**
 - Enumerate the causes of PPH.
 - Explain the prevention of PPH.

- Q.6.**
- Define gestational diabetes mellitus (GDM). **[1+3+6]**
 - Write about the effect of diabetes mellitus on pregnancy.
 - Explain the antenatal care of women with GDM.

OR

- Q.6.**
- Define the high risk pregnancy. **[1+3+6]**
 - How will you screen the high risk cases?
 - Explain the management of high risk cases.

Q.7. Answer **Any Two** of the following: **[2x5]**

- Obstetrical shock
- Puerperal sepsis
- Vacuum extraction